

Please note that with autoISF you are in an early-dev. environment, where the user interface is **not optimized for safety** of users who stray away from intended ways to use. Good safety features exist, but these are only as good as the development-oriented user understands and implements them. This is not a medical product, refer to disclaimer in [section 0](#)



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13.3 Methods involving simple Meal Announcement that might be stretched into a FCL
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[Available related case studies:](#)

Case study 13.1: Comparison 1 month FCL Automation vs autoISF
Case study 13.2: FCL using dynamicISF (call for an example so far un-answered)
Case study 13.3: FCL using Boost

13.1 Full Closed Loop using AAPS Master and Automations

AndroidAPS 3.0 was (Sep.2023) the first DIY system to launch Full Closed Looping as an option to manage T1D, if a described set of pre-requisites apply.

Key pre-requisites were described in

<https://androidaps.readthedocs.io/en/latest/Usage/FullClosedLoop.html>, and are sketched also in [section 1](#), with [case studies 1.1 – 1.5](#) underscoring the importance.

You may (not) have noticed: There was no big „marketing fuzz“ made around that FCL option. Seeing how many AAPS users struggle with even getting their basal, ISF and SMB settings right, it would be foolish to allure everybody to a supposedly very easy way of looping. True, it can be easy. But only after doing a personalized set-up project. Setting up is easier than what autoISF and the methods we get to in [section 13.3](#), demand, but still a project. It also requires a well mastered hybrid closed loop, to start from.

With attention to the pre-requisites, and avoiding extreme high carb diets, many (mostly: adult) users achieve satisfactory %TIR after supplementing AAPS Master with personalized Automations that attempt to strongly elevate iob upon recognition of a meal-related bg rise.

44 See also Case Studies, and the randomized cross-over study involving AAPS FCL: PubMed [First](#)
45 [Use of Open-Source Automated Insulin Delivery AndroidAPS in Full Closed-Loop Scenario:](#)
46 [Pancreas4ALL Randomized Pilot Study](#);

47

48 This method is **highly recommended for an entry into FCL for those who do not have the**
49 **interest, or lack the time, to deal with the very much more sophisticated and demanding other**
50 **routes** towards FCL, like autoISF, or also like the methods briefly presented below in [section 13.3](#).

51

52 Note that using the autoISF dev version of AAPS for this (*with "Enable ISF adaptation.." OFF*)
53 can be a good idea, to make use of features like SMB_range_extention and
54 SMB_delivery_ratios > 0.5. Compared to using AAPS Master, this allows stronger boosting of
55 SMB sizes, also when *not making use of autoISF*, but just of Automations, for FCL.

56

57 13.2 FCL using dynamicISF with AAPS or with Trio / iAPS

58

59 As opposed to

- 60 • autoISF, with it's bgAccel_ISF component , or to....
- 61 • AAPS Master, with Automations strengthening ISF at meal-related bg rises ...

62 dynamicISF was **not** designed to help boost SMBs asap after an omitted user bolus.

63 This is why methods like Boost ([section 13.3.1](#)) that even do have dynamicISF on bord, do extra
64 boosting routines for detected or announced meal starts.

65 Rather (as the name also suggests) it was designed to be used in hybrid closed looping to make
66 ISF react more dynamic to suspected swings in insulin sensitivity (which shows in bg values, and
67 in TDD trends). It does a similar job like Autosens, but can be much more amplified (by the users
68 tuning their dynamicISF adjustment factor (%)).

69

70 When using a fast insulin (and when some other pre-requisites discussed in section 1 are in place,
71 too), the dynamicISF method can be applied also to Full Closed Looping. (See [case study 13.2](#);
72 *not available by time of publication => This is a call for a dynISF FCL user to provide a case study*
73 *that contains a 1 week 24h scatter plot as well as one analyzed meal where we can see when and*
74 *how dynISF helped build iob, after not having bolussed*).

75

76 It will have a principal timing-disadvantage because responses are more tied to high bg values
77 than to acceleration (in autoISF) or to delta (in the Automations route to FCL).

78

79 On the other hand, people who 1) do have strong sensitivity swings and 2) cannot pro-actively
80 deal with those (e.g. by making profile switches) might be satisfied with the automatic (although a
81 bit late) adjustments that dynamicISF automatically will provide.

82

83 dynamicISF therefore could be characterized, in the FCL context, as a potential solution to a rather
84 care-free approach for those who do not seek best-possible performance (or who take other
85 measures, like low carb diet, to still reach pretty acceptable performance in FCL mode).

86

87 **More info** (caution, both not focussed on FCL:)

88 AAPS / search term dynamicISF in: <https://discord.gg/DfvK5HnxXu>

89 Trio or iAPS / section dynamic-isf-cr: <https://discord.gg/gGKXW5uX3m>

90

91

92 **13.3 Methods involving simple Meal Announcement that might be stretched** 93 **into a Full Closed Loop**

94

95 See also [section 7](#) on using autoISF in “MA” mode, involving a pre-meal bolus.

96

97 **13.3.1 Boost**

98

99 All of the additional code outside of the standard SMB calculation requires a daily time period
100 („Boost window“) to be specified within which it is active.

101 A variation of dynamicISF is used in which also predicted bg will be considered in varying degrees
102 (40...75%) to mimic the effects of higher insulin sensitivity at lower glucose levels.

103 When using Boost without carb inputs (permanent cob=0) a special **boosting of SMBs** is provided
104 when an **initial bg rise** is detected with a meal:

105 Boost uses the delta accelerations to drive an expectation of a higher future levels, allowing more
106 insulin to be delivered, so it principally can also work with no meal announcement (see [case study](#)
107 [13.3](#)). delta, short_avgDelta and long_avgDelta are used to trigger an early bolus (assuming IOB is
108 below a user defined amount).

109 This procedure goes in the direction of the bgAccel_ISF route discussed for autoISF
110 ([section 4.1](#)). If used with an excellent CGM, autoISF acceleration detection should be a bit
111 earlier, and boosting can be made much stronger in autoISF

112 For safety, the user sets a value of 2.5% (up to 5%) of TDD for the max. Boost Bolus (Boost Bolus
113 Cap).

114 For stronger boost, the default AAPS 50% SMB_delivery_rate can be overwritten with a higher in-
115 sulin percentage determined by the user. The SMB_delivery_ratio is called „Boost insulin required

116 percent“ here, and suggested not to go over 75%. The % can be defined variable with bg value
117 (like also in autoISF).

118 The Boost function automatically shuts off as soon as delta and the average deltas are aligned,
119 i.e. when the accelerated rise goes over into a constant rise (compare pp_ISF in autoISF).

120 However, the boost function is only „dormant“ if the boost window lasts longer for more meal-
121 related accelerations.

122 Additional functions are a step-count modified dynamic_ISF, inactivity detection etc

123 A couple of safety feature are integrated. The user can define an iob limit for boosts (like iobTH in
124 autoISF, here called UAM Boost max IOB in Preferences/Treatments) There is also a user adjusta-
125 ble Low Glucose Suspend threshold. This allows the user to set a value higher than the system
126 would normally use, such that when predictions drop below this level (65...100), a zero TBR is set.

127 Application example, see [case study 13.3](#)

128 More info: <https://discord.gg/nYC4T9PgCR> ; <https://github.com/tim2000s/no-bolus-dev>
129 : https://github.com/tim2000s/Boost_AAPS_3.2/blob/Boost-Master-3.2/README.md

130 Contact: Tim Street @ diabettech.com

131

132 **13.3.2 AIMI** (available on AAPS and on Trio platform)

133

134 AIMI has a single goal: to minimize the decisions necessary to maintain the target range, simplify
135 the composition of the profile for the user or doctor accompanying the patient, and allow the patient
136 to live normally without having to count carbohydrates or even without signifying physical activity
137 (especially for brisk walking).

138 A key component of AIMI concept is to give a **small pre-bolus before each meal** (“Meal
139 Announcement” that also provides some pos. iob).

140 • A **simplified profile** composition (neutral ISF around 100, DIA 9, target 90-90, a single value for
141 basal, a ratio that is not used in AIMI, so not important) For a first basal estimate, you can use the
142 TDD / weight ratio.

143 • Some variables in preferences that are important (AIMI_UAM which allows AIMI to make
144 decisions, Max SMB size which is the highest value for an SMB, B30_duration (which is the
145 duration during which the **basal will be forced after a manual bolus**), B30_upperBG and
146 B30_Upperdelta (these last two variables represent the conditions for replacing smb with a
147 consistent TBR depending on the delta)

148 • The basal profile is calculated by a polynomial equation.

149 • The ISF is calculated from the TDD (**dynamicISF**) and is adjusted based on the evolution of TIR
150 throughout the day and the **detection of physical activity**.

151 • The detection of glycemic rise (or the opposite situation) is also calculated by a polynomial
152 equation, which will influence the change of target but also the replacement of SMB by a TBR
153 between 100% and 500% or by an SMB of the same equivalence.

154 • SMB calculation is done in several ways specific to AIMI depending on the evolution of the delta
155 and IOB, with a distribution that can be done in three parts depending on the conditions.

156

157 Example scenario of execution, on almost all existing variants:

158 1. Make a "standard" manual bolus. I usually do 1.5U or 2U with luymjev

159 2. Just after this bolus, AIMI will force the 500% TBR for a duration defined by the user. The
160 observation made is that the absorption of insulin such as humalog for example is acceler-
161 ated and will strongly limit the first wave.

162 3. Depending on the options chosen, it is possible to receive an SMB of the initial manual bo-
163 lus size after the duration of the 500% TBR

164 4. Then the rest of the calculations will depend on the result of a polynomial equation and its
165 evolution.

166 5. A few hours later, if the patient decides to take a walk to go shopping, or other activities re-
167 quiring movement, the phone sensor will send information on the number **of steps taken**.
168 This will result in a reduction of the profile to about 60%. The return of the profile to normal
169 will be done in stages, in the first half hour following the activity, the profile will be restored
170 to about 80%.

171 The AIMI developer has been working on incorporating machine learning (using tensorflow lite).

172 More info <https://discord.gg/jnDnJtcBb7>

173

174 The developer hasn't kept the code public. AIMI can only be obtained as an apk via joining their
175 WhatsApp group or here:

176 https://github.com/MTR93600/OpenApsAIMI/tree/dev_mergemilos_addOAPSAIMI

177

178 Given the very high number of changes happening in this AAPS variant, it is probably deemed
179 important to keep it in a tight sub-community. But, caution: This can be seen as violation of the
180 Open Source principle

181

182 Contact: Mathieu Tellier @ AndroidAPS User FB / Twitter @MTR93600/, Discord: MTR

183

184 13.3.3 EatingNow (EN)

185

186 This version of AAPS has evolved over time using elements from AIMI and Boost. It includes a
187 modified dynamicISF which moves ISF modulation in the direction as pioneered by autoISF, and
188 also uses Automations for FCL.

189 "Eating Now" (EN) allows user definable SMB's when deltas are sufficient and accelerating.

190 The intent of this plugin is the same, to deliver insulin earlier using mostly the AAPS predictions.

191

192 As all other variants for FCL, also EatingNow requires to set glucose TT occasionally, to nudge the
193 loop in certain direction, notably to announce and be prepared for exercise.

194

195 Operating Modes provide 3 levels of „aggressiveness“ in 3 time windows:

- 196 • Master AAPS w/up to 120 min basal per SMB when EN is off (usually set for night-time).
- 197 • EN (usually set for daytime) is when the modified algorithm is capable of boosting ISF and
198 insulin delivery. At BG level rises within the EN Window, a „UAM maxBolus“ is given as a
199 first SMB. Recommended Setting: 1h current basal in units (max allowed: 2).
- 200 • ENW: A further boosted SMB will be issued in this ENW time window (e.g. for breakfast, or
201 generally for the first meal of a day, after fasting, with higher insulin need). Upon detection
202 of rising glucose, a SMB called Breakfast COB maxBolus is given by the loop. Recom-
203 mended Setting: 25% of average breakfast total units

204

205 EN uses the dynamicISF concept, modified to making ISF stronger with increased eventualBG
206 predictions.

207

208 Specifically for the ENW (usually: breakfast window), an additional boost factor called Breakfast
209 ISF/CR Percentage (e.g. 125 or 150%) can be applied

210 A setting „TIRS“ provides a very simple version of autoISF (dura_ISF) and sharpens ISF
211 temporarily when bg „seems stuck“ above a certain value.

212

213 Autosens sensitivityRatio will be overridden by EN sensitivity options.

214

215 SMB delivery ratio for insulinReq. is set to 65% for when EN is disabled (overnight, usually).

216 It is recommended to set maxSMBBasalMinutes and maxUAMSMBBasalMinutes to 30 minutes

217 max as these will be used when EN is OFF or in SLEEP mode. Falling back on OpenAPS SMB

218 settings is considered as the safe mode, should you experience any issues with sensitivity or EN

219 settings in general

220 It is set 85% for an active ENW, or 75% when EN is on but ENW not active

221

222 Furthermore, SMB optionally can be disabled day/night below defined bg level/s (SMB Disabled)

223 **More info** <https://discord.gg/XqhnPRChEP> (method description in pinned post)

224 <https://github.com/dicko72/AAPS-EatingNow> scroll down to README.md

225 Contact: dicko via Discord channel

226

227 **13.3.4 Tsunami**

228

229 The Tsunami loop algorithm analyses blood glucose and insulin activity developments to estimate
230 bolus requirements during meals, without the necessity of carb announcements.

231

232 Users must make a **meal announcement via a button** on AAPS main screen. It switches on the
233 main Tsunami algorithm for a finite amount of time.

234

235 In between meals (when Tsunami is inactive), users are given the choice between running a
236 weaker version of the Tsunami algorithm (called wave), or falling back tooref1.

237

238 A “historic” merit of this method was that it pioneered a BG smoothing algorithm that later
239 became included as a plugin in AAPS.

240 The insulin models dynamically readjust DIA based on bolus size so that a user-set, fixed
241 DIA value is no longer needed.

242

243 For best results, it is recommended to issue a **bolus** at the beginning of a meal to account for the
244 disadvantageous kinetics of subcutaneously administered insulin in a UAM setting.

245 **More info** <https://discord.gg/veRKcgwVUT> GitHub repository: <https://github.com/piecycle/tsunami>
246 [official documentation: https://cdn.discordapp.com/attach-](https://cdn.discordapp.com/attachments/969948954949189633/972852790739238992/tsunami_guide_3_2.pdf)
247 [ments/969948954949189633/972852790739238992/tsunami_guide_3_2.pdf](https://cdn.discordapp.com/attachments/969948954949189633/972852790739238992/tsunami_guide_3_2.pdf)

248

249 Contact: nichi#1391 on discord / piecycle on GitHub

250

251 **13.4 No-Bolus Looping with Carb Entries**

252

253 Some oreof(1) loopers attempting to go full closed loop reported that they do best when they (do not
254 bolus but) give their loop precise carb (and absorption time) information. This:

255 * announces a meal to follow (so it is not UAM, but might be called full closed looping if the
256 insulin management is left 100% to the loop)

257 * provides data on cob, and with the glucose and insulin activity info the loop has, it can
258 always calculate how much more carbs are to become absorbed (to the extent the carb-
259 related infos the user put in is correct)
260 * will display realistic cob info to the user, including cob info looking forward (rather than
261 only calculating carb deviations for the past minutes or hours, and making some coarse
262 assumptions for the upcoming hour). It gives the user better feeling of safety if she/he can
263 see cob info in addition to the available iob info, and insulin activity prediction.
264

265 With detailed carb (amounts + absorption times) inputs, the loop has best-possible info to provide
266 „the best expert fit“ of insulin activity and carb absorption.

267 It still rarely can come close to physiological values, because the time-delays inherent in
268 our „artificial pancreas“, notably the stretched out DIA, make it difficult still, compared to a
269 real pancreas.
270

271 So, carb inputs could help. However,

- 272 • only to the extent amounts and time pattern for absorption („eCarbs“) are correct ((which,
273 every day, is pretty much a mission impossible))
- 274 • theoref(1) loop still largely „waits for glucose to rise“, and there is no significant time ad-
275 vantage from inputting carb info

276 Only the **user**-bolussing *for expected* carb absorption in hybrid closed loop offers a
277 convincing time advantage (but with associated risks).

- 278 • inputs require actually more attention to detail than it is good practice even in AndroidAPS
279 hybrid closed loop, so in that respect a step back, not forward.

280 Entering **precise** carb information takes away a very large part of the attractiveness of full closed
281 looping.

282 And entering *imprecise* carb info could easy be inferior to not doing *any* carb inputs = to letting the
283 *UAM mode* oforef(1) figure out further carbs that probably come to be absorbed in the next
284 minutes, judging from the pattern of the calculated past *carb deviations* (see [section 4.5](https://openaps.readthedocs.io/en/latest/docs/While%20You%20Wait%20For%20Gear/Understand%20-determine-basal.html#understanding-the-basic-logic-written-version) and
285 [https://openaps.readthedocs.io/en/latest/docs/While%20You%20Wait%20For%20Gear/Understand
286 -determine-basal.html#understanding-the-basic-logic-written-version](https://openaps.readthedocs.io/en/latest/docs/While%20You%20Wait%20For%20Gear/Understand%20-determine-basal.html#understanding-the-basic-logic-written-version)).

287 PS: Because that is so, also loopers who do carb inputs get the UAM predictions besides their other
288 predictions, and their algo makes a judgement (every 5 minutes) as to what the best calculation might be
289 for where glucose, underlying „real“ carb absorption, and estimated carb deviation are headed.
290
291
292

293 A very recent experimental development is complementing autoISF with “**autoCR**”, CR standing for
294 carb ratio.

295 The developer and pilot user (JohnKitching — 21.02.2026 10:07 @ <https://discord.gg/t6Vjep5e>) reports
296 improvements over his autoISF FCL if using that new modality, and making carb inputs. (No user
297 bolussing is required, so this is pretty close to a FCL).

298 Expect autoCR implementation offered as an option in a dev variant of iAPS, starting around
299 Q2/2026. <https://github.com/Artificial-Pancreas/iAPS/pull/1787>

300

301 13.5 Machine Learning

302

303 Involving machine learning (“artificial intelligence”) could help both in the learning/tuning phase, but
304 also in fine adjustments in daily utilization.

305

306 The study “The artificial pancreas and meal control” that was already referenced ([section 1.2](#) , or
307 [https://github.com/bernie4375/HCL-Meal-Mgt.-ISF-and-IC-settings/blob/HCL-.settings-main-repo-](https://github.com/bernie4375/HCL-Meal-Mgt.-ISF-and-IC-settings/blob/HCL-.settings-main-repo-(pdf))
308 [\(pdf\)](#)) discusses *on page 80* the application of machine learning in some predictions of postprandial
309 glucose response (IEEE Control Systems Magazine, ResearchGate: The Artificial Pancreas and
310 Meal Control. A. El Fathi et al, IEEE Control Systems Magazine Feb.2018 p.67-85.).

311 So there is already a body of data and evidence. To which extent it lends itself to UAM remains to
312 be researched. For this, lots of data would have to be captured from UAM loopers, and I fear many
313 more data would be required than what could easily be captured in Clarity® or even the AAPS
314 logfiles.

315

316 In the DIY universe, a prototype solution (to some aspects) was already developed for AIMI
317 ([section 13.3.3](#)).

318

319 We might see industry come up with a 1st generation solution that will probably be geared to folks
320 with miserable HbA1c and poor carb counting/meal handling, to offer a safe gradual improvement.

321

322 A top performing entirely self-learning system might be impossible to design:

323 For instance, if today you do something entirely different from yesterday (don’t we all want this
324 freedom – even need it? Think about the fasting day following a feasting day...) there are
325 two problems:

- 326 • Such systems rely on information from the preceding day, or an average of several preced-
327 ing days
- 328 • The user does not know/learn much about how the system works, what it is calibrated for
329 today, how she/he might intelligently change something for the specific different situation
330 coming up. This seems like the opposite of the FCL solutions we discussed, for instance

self-defined Automations, combined with profile switches for to-be-expected temporary sensitivity shifts (see [section 13.1](#) , or the more sophisticated options presented for exercise in [section 6.1.3](#) and [6.2](#)).

13.6 Dual Hormone Systems

Off-topic: Besides using a glucagon analogue as a second independent hormone, there is also research on just adding very small amounts into the (one) insulin cartridge. Then it seems to work like an additive to make Lyumjev even faster acting, which would be a good thing notably for FCL.

Brief discussion and further reference on this topic see in “Insulins, DIA...pdf” at:

<https://github.com/bernie4375/HCL-Meal-Mgt.-ISF-and-IC-settings>

This is not our topic in this section, though.

Many see a **dual hormone full closed loop** as the ultimate system.

The beauty of this concept would be that the second (cartridge in the) pump could influence the glucose curve via giving glucagon or an analogue, thus overcoming the strongest limitation our current systems have:

Taking basal away (zero-temping) is only a severely limited course of action against impending hypoglycemias, and therefore, to keep things safe at the back-end of each meal, fighting glucose highs is more limited than we would like to see.

In conclusion, the glucagon component **not only helps stay out of hypos. It enables a more aggressive treatment for preventing, or reducing, high glucose values, as well.**

While insulin and carbs have complex activity curves stretching over hours, glucagon has a window of physiological activity starting 5-10 minutes after administration, and lasting only 30-40 minutes. Compared to insulin and carbs, that makes it a better component for rapid corrections (without a lengthy “tail” of action).

As glucagon does not per se introduce more calories, but stimulates glucose release from the liver, there should at least be no concern about gaining body weight from eventual roller-coasters the dual loop might send us into. Actually there could be a nice side benefit of helping in body weight control. Also, activity/sports management could become as easy as the meal management became in the UAM step into full closed looping.

It will be interesting to see for which application(s) the dual loop will be developed and launched; as part of a full closed loop with top performance, or as part of even only a hybrid closed loop for problem patients?

368 It remains to be seen how well such systems work in day-to-day circumstances. And whether “real
369 people” will be able to handle all the involved technology, and use it in ways that truly could justify
370 the substantial extra cost.

371

372 The author currently is not really looking forward to become loaded with even more technology,
373 and quite happy with an aggressively tuned full UAM closed loop (...and an occasional nice post-
374 dinner, or before/during-exercise snack).

375

376 However, the dual hormone path holds enough promise to learn more about it, and to test it some
377 time in the near future.

378 13.7 Co-Medications that might facilitate FCL

379

380 Although the hype around GLP.1 and GIP receptor agonists (Tirzepatide, Mounjaro® and
381 others) is about weight loss, and in most markets type1 diabetics are not even eligible to
382 get a prescription, there is the interesting hypothesis, that **slowed digestion makes the**
383 **job significantly easier for a full closed loop**. More importantly, it could lower the
384 hurdles for fine-tuning, and open up an avenue to use FCL to a broad population of T1Ds.

385

386 Aside from the FCL aspect, the tendency towards improved %TIR in the T1D population-
387 was demonstrated in a few smaller studies already, e.g.:

388 **Efficacy and Safety of Tirzepatide in Adults With Type 1 Diabetes:** A Proof of Concept Obser-
389 vational Study (J Diabetes Sci Technol 2025 Mar;19(2):292-296; PMID: **38317405** PMCID:
390 [PMC11571402](#) DOI: [10.1177/19322968231223991](#)) [Halis Kaan Akturk](#)¹, [Fran Dong](#)¹, [Janet K](#)
391 [Snell-Bergeon](#)¹, [Kagan Ege Karakus](#)¹, [Viral N Shah](#)²

392 **Background:** Tirzepatide is approved by the United States Food and Drug Administration (FDA)
393 for the management of type 2 diabetes. The efficacy and safety of this drug have not been studied in
394 people with type 1 diabetes (T1D).

395 **Methods:** In this single-center, retrospective, observational study, hemoglobin A1c (HbA1c),
396 weight, body mass index (BMI), and continuous glucose monitoring (CGM) data were collected
397 from electronic health records of adults with T1D at initiation of tirzepatide and at subsequent clinic
398 visits over 8 months. Primary outcomes were reduction in HbA1c and percent change in body
399 weight and secondary outcomes were change in CGM metrics and BMI over 8 months from base-
400 line.

401 **Results:** The mean ($\pm$ SD) age of the 26 adults (54% female) with T1D was 42 ± 8 years with a
402 mean BMI of 36.7 ± 5.3 kg/m². There was significant reduction in HbA1c by 0.45% at 3 months
403 and 0.59% at 8 months, and a significant reduction in body weight by 3.4%, 10.5%, and 10.1% at 3,
404 6, and 8 months after starting tirzepatide. Time in target range (TIR = 70-180 mg/dL) and time in
405 tight target range (TTR = 70-140 mg/dL) increased (+12.6%, $P = .002$; +10.7%, $P = .0016$, respec-
406 tively) and time above range (TAR >180 mg/dL) decreased (-12.6%, $P = .002$) at 3 months, and
407 these changes were sustained over 8 months. The drug was relatively safe and well tolerated with
408 only 2 patients discontinuing the medication.

409 **Conclusions:** Tirzepatide significantly reduced HbA1c and body weight in adults with T1D.

410

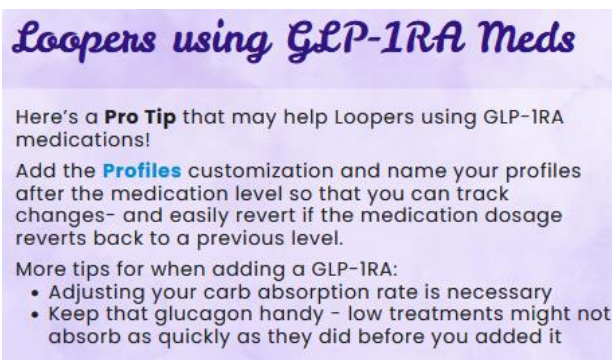
411 Follow the discussion regarding utility in our diabetes management e.g. here:

- 412 • Discord: <https://discord.gg/YvrHYmdamc>
- 413 • Diabettec: <https://www.diabettech.com/glp1-ra/glp1-ras-and-type-1-diabetes-a-retrospective-study-of-incretin-based-therapy-use-with-open-source-aid/>
- 414

415

416

- 417 • A tip from Loop-and-Learn Newsletter Jan.2026:



418

419

420

421 **Call for a case study submission if someone was able to test this.** *Note: The author had*

422 *proposed a study but could not get a prescription to actually do it. See Discord.*

423

424

425

426 Final Remark

427

428 This is an exciting time to be part of the open source T1D community. Anyone is welcome to

429 contribute ideas, help develop software or instructions how to use.

430 Carefully weigh for yourself what may be your entry point for eventually surmounting the initial

431 hurdles, and **JUST EAT happily ever after.**

432